

SPINE EXPERT INDIA

National Spine Care Intelligence Network

SEI-SCT v1.0 · Final Edition
Minimum Viable Clinical
Documentation Layer (MVCDL)

Structured Specialist Consultation & Clinical Decision Template

For Doctor Use · In-Consult Touchscreen Form · Generates Patient Prescription & Care Pathway

Purpose: This is a structured clinical decision form, not a free-text consultation note. It converts your clinical judgement into discrete, registry-ready data that drives the AI-generated patient care plan, prescription, and outcome tracking — while preserving full surgeon autonomy over diagnosis and pathway selection.

- ✓ Completes in <2 minutes
- ✓ Reduces missing clinical data
- ✓ Maintains specialist autonomy
- ✓ Drives AI patient pathway
- ✓ Registry-ready clinical data
- ✓ Enables audit & QI

Patient Intelligence → Specialist Assessment → Clinical Decision → Care Pathway → Outcome Tracking

SECTION 1 — Patient & Consultation Details

Identifiers and visit context · Auto-linked to pre-consult record

Patient ID: _____ AUTO-GENERATED Doctor: _____ Date: _____

Consultation Type

- ☐ First Consultation
- ☐ Follow-up
- ☐ Second Opinion
- ☐ Post-operative Review

SECTION 2 — Pre-Consult Intelligence Validation

AI-generated from patient questionnaire (SEI-PQ) · Reviewed by doctor

Spine Expert India Pre-Consult Summary

AI-GENERATED

Patient Risk Category (from SEI-Score)

- ☐ Low Risk
- ☐ Moderate Risk
- ☐ High Risk

AI Suggested Pathway

- ☐ Education / reassurance
- ☐ Conservative pathway
- ☐ Specialist evaluation
- ☐ Priority review

Doctor Validation

MANDATORY

After consultation, the pre-consult information was:

- ☐ Clinically accurate
- ☐ Partially accurate
- ☐ Needs correction

Comments (optional)

Free text — doctor remarks on pre-consult accuracy

SECTION 3 — Primary Clinical Problem

Doctor-confirmed presenting complaint

Main Complaint

- ☐ Neck pain
- ☐ Back pain
- ☐ Arm pain / cervical radiculopathy
- ☐ Leg pain / sciatica
- ☐ Numbness / tingling
- ☐ Weakness
- ☐ Walking imbalance
- ☐ Deformity
- ☐ Post-operative problem
- ☐ Other

Duration of Symptoms

- ☐ < 6 weeks
- ☐ 6 weeks – 3 months
- ☐ 3 – 6 months
- ☐ > 6 months

SECTION 4 — Clinical Risk Assessment

Red flag screening · Determines urgency category

Neurological Status

Motor weakness

- ☐ No
- ☐ Present

Sensory deficit

- ☐ No
- ☐ Present

Bladder / bowel involvement

- ☐ No
- ☐ Present

Gait imbalance

- ☐ No
- ☐ Present

Overall Risk Category

MANDATORY

- ☐ Routine spine care — no red flags, no urgent pathology
- ☐ Needs specialist follow-up — persistent or evolving symptoms
- ☐ Urgent evaluation required — any red flag or progressive deficit

SECTION 5 — Clinical Assessment

Working diagnosis and clinical remarks

Working Diagnosis

- ☐ Non-specific mechanical spine pain
- ☐ Disc-related disorder
- ☐ Radiculopathy
- ☐ Degenerative spine disease
- ☐ Spinal stenosis
- ☐ Cervical myelopathy
- ☐ Spondylolisthesis
- ☐ Deformity / scoliosis
- ☐ Trauma
- ☐ Infection
- ☐ Tumour suspicion
- ☐ Other

Clinical Diagnosis / Remarks

Short free text — the only narrative field in this section

Free text — concise clinical diagnosis statement

SECTION 6 — Clinicoradiological Correlation

Imaging review and interpretation

Imaging Available

- ☐ None
- ☐ X-ray
- ☐ MRI
- ☐ CT
- ☐ Other

Imaging Interpretation

- ☐ Symptoms correlate with imaging
- ☐ Partial correlation
- ☐ Incidental findings
- ☐ Imaging insufficient

Further investigation required?

- ☐ No
- ☐ Yes

If yes, reason:

- ☐ Diagnosis confirmation
- ☐ Neurological assessment
- ☐ Treatment planning
- ☐ Red flag evaluation

⚠ Physical Examination Required by Specialist USE WHEN DIAGNOSIS CANNOT BE REACHED REMOTELY

Select when teleconsultation / available information is insufficient to safely confirm a diagnosis, and urgent in-person assessment is needed.

- ☐ Not required — diagnosis reached
- ☐ Required — physical examination mandatory before diagnosis / pathway can be finalised

If required, reason

- ☐ Inconclusive remote findings
- ☐ Suspected red flag needing hands-on confirmation
- ☐ Imaging / history mismatch
- ☐ Other

Patient notice (auto-generated): Your specialist has reviewed your information and determined that an in-person physical examination is required before a diagnosis can be confirmed. Please book a physical examination visit at the earliest available slot.

SECTION 7 — Final Care Pathway Decision

Mandatory selection · One primary pathway drives the AI patient plan

PATHWAY 1 — Education & Reassurance

☐ Selected

For patients where: no red flags · no urgent pathology · education is the primary treatment

PATHWAY 2 — Conservative Spine Care

☐ Selected

Physiotherapy components

- ☐ Core strengthening
- ☐ Cervical stabilisation
- ☐ Mobility exercises
- ☐ Nerve mobilisation
- ☐ Postural correction
- ☐ Walking programme
- ☐ Other

Lifestyle advice

- ☐ Activity continuation
- ☐ Ergonomic modification
- ☐ Weight management
- ☐ Exercise programme
- ☐ Workplace modification

PATHWAY 3 — Further Evaluation

☐ Selected

Reason: Persistent symptoms · Failed conservative care · Diagnostic uncertainty · Complex case

PATHWAY 4 — Surgical Evaluation

☐ Selected

Reason: Progressive neurological deficit · Structural compression · Instability · Deformity · Failed appropriate conservative care

PATHWAY 5 — Urgent Intervention

☐ Selected

Reason

Free text — urgent intervention rationale

SECTION 8 — Investigations Advised

Only when clinically indicated

Laboratory Investigations

SELECT IF REQUIRED

- ☐ CBC
- ☐ ESR
- ☐ CRP
- ☐ Vitamin D
- ☐ Calcium profile
- ☐ HLA-B27
- ☐ Rheumatological profile
- ☐ Blood sugar / HbA1c

Other: _____

Radiological Investigations

X-ray Spine

Region

☐Cervical☐Thoracic☐Lumbar

Views: _____

MRI Spine

Region

☐Cervical☐Thoracic☐Lumbar

With

☐Screening☐Contrast

☐CT Spine☐CT Myelogram☐EOS / Long cassette X-ray

Other: _____

SECTION 9 — Prescription Input

Only free-text section in this form

Diagnosis: _____

1. Medicines Advised

Handwritten / free text note — doctor's own prescription format

Free text — medicine name, dose, duration, instructions (handwritten note area)

2. Pain Management Advised

☐Oral medication only☐Spinal injection — epidural☐Spinal injection — nerve block☐Spinal injection — facet joint☐Transdermal patch☐Topical application
☐Heat / cold therapy advised☐Other

Details / site / frequency: _____

SECTION 10 — Referral Pathway

Where the patient should go next, if outside this specialist's scope

1 Referral to Allied Specialist

☐Neurologist☐Rheumatologist☐Knee specialist☐Shoulder specialist☐Physiotherapist☐Other speciality

Speciality / name (if other): _____

2 Review for Physical Examination — Decision-Making

For cases needing in-person assessment before a final pathway decision

☐Refer to nearest Spine Expert India network doctor for physical examination

City / preferred location: _____

3 Surgery Advised at Earliest

If patient is away from this specialist's location

☐Meet nearest spine surgeon☐Choose from Spine Expert Network physical doctors via Spine Expert app

City / preferred location: _____

Patient notice (auto-generated): Surgery has been advised at the earliest. If you are away from this clinic, please select a spine surgeon from the Spine Expert India network nearest to you, using the Spine Expert app.

SECTION 11 — Follow-up Plan

Review timing and consultation mode

Review

☐No follow-up required☐2 weeks☐6 weeks☐3 months☐6 months

Follow-up Mode

☐Teleconsult☐Physical consultation☐Regional Spine Expert Centre

SECTION 12 — Outcome Registry Baseline

Automatically captured from pre-consult PROM scores

VAS Pain Score

0–10 scale

/ 10

ODI — Oswestry

Disability index

%

NDI — Neck Disability

Disability index

%

mJOA

Myelopathy score

/ 18

Quality of Life

Self-rated

/ 10

Work Limitation

☐None☐Partial☐Unable to work

Strategic Registry Fields

MANDATORY — LONGITUDINAL METRICS

These two fields are mandatory for every consultation. Tracked over time, they demonstrate appropriate imaging utilisation and ethical, evidence-based surgical decision-making across the network.

Was MRI required at this stage?

☐Yes☐No

Was surgery indicated at this stage?

☐Yes☐No

AI-GENERATED PATIENT OUTPUT — "YOUR SPINE CARE PLAN"

AUTO-GENERATED FROM ABOVE

#	Component	Source
1	Understanding your condition	Section 5 diagnosis
2	Why this treatment pathway was selected	Section 7 pathway
3	Medicines prescribed	Section 9
4	Pain management plan	Section 9
5	Physiotherapy / exercise plan	Pathway 2 components
6	Lifestyle recommendations	Pathway 2 lifestyle advice
7	Referral pathway / next steps	Section 10
8	Expected recovery timeline	Diagnosis + pathway
9	Warning symptoms requiring urgent review	Section 4 risk flags
10	Follow-up schedule	Section 11

BEHIND-THE-SCENES REGISTRY DATA FLOW

Patient Profile

Clinical Diagnosis

Treatment Pathway

Investigation Decision

PROM Baseline

Outcome Tracking

Every completed consultation contributes one structured record to the **Spine Expert India Clinical Intelligence Registry** — the national dataset underpinning audit, AI model improvement, and population-level spine health insight.

FIVE CLINICAL PATHWAYS — QUICK REFERENCE

Pathway	Colour	Focus
Pathway 1	GREEN	Education & reassurance
Pathway 2	BLUE	Conservative care
Pathway 3	YELLOW	Further evaluation
Pathway 4	ORANGE	Surgical evaluation
Pathway 5	RED	Urgent intervention